

	EMERGENCY MEDICAL SERVICES AGENCY	POLICY NUMBER	716
	A Division of the Merced County Department of Public Health	Effective Date	11/2025
Authority: Health and Safety Code, Division 2.5, and California Code of Regulations, Title 22, Division 9, Chapter 4, 1797.220		Initial Date:	01/2017
		Review Date:	01/2028

NAUSEA AND VOMITING	
May be given prior to transport to prevent nausea/vomiting. May be given to prevent nausea/vomiting from pain medication administration. May use Diphenhydramine as an alternative if <u>allergic</u> to Ondansetron.	
ADULT	PEDIATRIC
BLS	
Assess Vitals / Obtain SpO ₂ / Oxygen. Titrate to SpO ₂ of 94% or higher / Secure Airway - Assist Ventilations, if needed. Check Blood Sugar – If indicated.	
ALS	
Follow BLS procedures if applicable Obtain 4-Lead ECG – If indicated. Vascular Access – If indicated - IV/IO rate as appropriate If the patient has severe vomiting, signs of dehydration, or hypotension – Refer to EMS Policy #738 – Non-Traumatic Shock	
Ondansetron - 4 mg ODT/IM or slow (over 1 minute) IV/IO - Attempt to administer ODT as first route of choice if possible - May repeat every 10 minutes if necessary - Max dose 12 mg - Do not use ODT with GCS < 14	Ondansetron - 2 - 4 mg ODT/IM or slow (over 1 minute) IV/IO - Attempt to administer ODT as first route of choice if possible - May repeat every 10 minutes if necessary - Max dose 12 mg - Do not use ODT with GCS < 14
Diphenhydramine - 50 mg PO/IM OR 25 mg IV/IO May repeat once IV/IO after 30 minutes to a max dose of 50 mg	Diphenhydramine - 1 mg/kg PO/IM/IV/IO - Max dose 25 mg - Single dose
Special Considerations	
Syncope may occur when administering IV Ondansetron - Administer slowly over one (1) minute. Ondansetron is safe for patients who are pregnant or breast feeding. For <u>prolonged transport time</u> , may consider Diphenhydramine for motion sickness.	

Signatures on File

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